

SPMM
Smart
Revise

Psychotherapy

Paper B

Syllabic content 9

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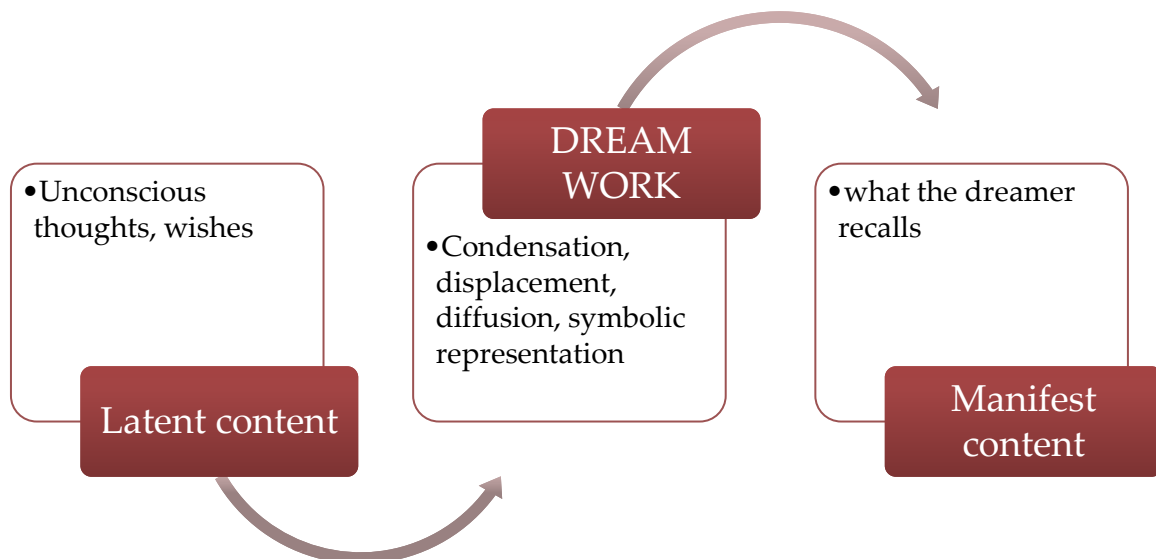
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1. Dynamic Psychotherapy

Development of psychodynamic theory

Central concepts of Freudian thought

- ★ **Repression:** unacceptable ideas, memories and thoughts are pushed out of awareness, into the unconscious.
- ★ **Parapraxes and dreams:** Parapraxes (Freudian slips) and dreams give clues as to what is in the unconscious. Parapraxes are considered to be due to the *return of the repressed* materials that slips out as words during conversation.
- ★ **Free association:** The patient is encouraged to say whatever comes into his mind, however fleeting or trivial. This helps to reveal aspects of the unconscious mind.
- ★ **Dreams:** Made up of: unconscious mental matter, residues from the day, and stimuli experienced during sleep (e.g., heat)



- ★ **Dream work** turns latent into manifest content and includes the following processes:
 - **Condensation:** two or more unconscious impulses are combined into a single image. E.g., a strict father and a punitive teacher combine in the dream into one frightening monster
 - **Diffusion or Irradiation:** one unconscious impulse is represented by several images (the opposite of condensation)
 - **Displacement:** the energy invested in one object or idea gets transferred to another. E.g., a wishful phantasy about murdering one's father becomes represented by shooting a teacher

- **Symbolic representation**: an innocent or less highly charged image is used in the place of something that is potentially too overwhelming. E.g., a wishful fantasy about shooting one's father becomes an image of hunting a stag
- ★ **Instincts**: Freud maintained that the mind developed in order to manage our instincts.
 - Destructive, aggressive instincts (part of **Thanatos**: the Death instinct)
 - Life-affirming libidinal/sexual instincts (considered part of **Eros**: the Life instinct).

The pressure that arises from the instincts pressing to be fulfilled leads to anxiety.

Models of Mind

Freud tried to explain how the mind works by using Models of the Mind. First was the Topographical Model; this was later superseded by the Structural Model.

★ Topographical Model of the Mind

According to this, the mind is divided into three regions:

- The *unconscious*
 - Includes repressed memories, sensations and impulses
 - Governed by the Pleasure Principle: the instincts seek out gratification
 - Characterised by Primary Process thinking: defies logic, not restricted by reality
- The *preconscious*
 - At the interface between the Unconscious and the Conscious
 - Maintains a 'repressive barrier' that censors unacceptable wishes and desires
- The *conscious*
 - Linked in with the reality of the outside world
 - Characterised by Secondary Process thinking: bound by time and space

★ Structural Model of the Mind

- Id: full of the instinctual aspects of the individual (e.g., sexual and aggressive impulses), mostly unconscious
- Ego: the executive organ of the mind- linked in with reality
- Superego: the seat of internalised morals and values. Can be quite punishing ('thou shalt not...') or helpful in striving for a goal (the Ego Ideal).

Freud's Psychosexual Stages of Development

Oral (0 to 18 months approx.): The *mouth and sucking* are the infant's focus. This is no surprise as it promotes feeding and hence the baby's survival. A fixation on this phase in later life may lead to difficulties such as alcoholism or excessive eating.

Anal (18 months to 3 years approx.): This is the period around potty training. The infant becomes able to control the *function of the anal sphincter*, and can therefore decide when to pass stool, and when not to. This sense of power and control is expressed in the term 'anally retentive',

indicating that a person's character is controlling. If this fixation persists in adulthood it can manifest in disorders such as OCD (very controlling).

Phallic/Oedipal (3-5 years approx.): The *genitals* become of interest and there is the differentiation between boys and girls. This stimulated the feelings described in the Oedipus Complex, and it is proposed that its resolution leads to the formation of the Super-Ego, with introjection of parental values (the prohibition of patricide and incest).

The Oedipus Complex: Based on Greek mythology, it proposes that in a male infant/young child there is a wish to kill off the father, in order to marry the mother. This is not as strange as it seems: the experience of competition between younger men towards older men and a wish to usurp them is quite common, as is a boy's wish to have his mother all to himself and not have to share her. In later life a man may choose a wife who has similar qualities to his mother, over whom he also feels possessive.

Freud described the opposite dynamic for girls: **the Electra complex**. He suggested that girls realise they and their mothers don't have a penis; they feel they want one (*penis envy*); they turn towards the father (and later in life to other men) in order to get a penis and have a baby. This thinking has been quite heavily criticised and Freud admitted not understanding women very well.

Neo Freudians

★ Melanie Klein

- Through her study of early life and children's play, Melanie Klein proposed that aggressive and destructive forces were central components of early development.
- Paranoid-schizoid Position. In this position the world is split into 'good' and 'bad'. The infant has lots of destructive feelings and thoughts about a 'bad' mother; there is a fear that the 'bad' mother will retaliate and punish the infant (this is the 'paranoid' component). One way for the infant to deal with it is by retreating and cutting off (this is the 'schizoid' component)
- Depressive position. Once an infant is able to integrate good and bad, and see the mother as having both qualities, then s/he may begin to feel guilty and wish to repair any damage caused.

★ Carl Gustav Jung

- Founded the school of Analytic Psychology.
- Jung's psychic apparatus:
 - **Collective Unconscious:** mankind's collective symbolic past, which includes **Archetypes** (representational images of universal symbolic meaning e.g., the Hero, the Old Wise Man etc.)

- **Personal Unconscious:** an individual's unconscious, comprising Complexes (sets of ideas and feelings triggered by interpersonal interactions)
- Ego: individual's conscious mind
 - **Other Jungian concepts:**
 - **Persona:** a mask covering one's personality, but what is shown to the outside world
 - **Anima:** the unconscious feminine aspect of a man
 - **Animus:** the unconscious masculine aspect of a woman
 - **Shadow:** a personification of the less acceptable aspects of oneself
 - **Individuation:** the process in which the individual develops self-identity
 - **Extraversion** (outgoing) and **Introversion** (keeping to oneself)

★ **Winnicott**

- According to Winnicott, children's psychological development occurs in a zone between reality and fantasy called **transitional zone**. Play is an important aspect of development of a child. Parental control and impositions can lead to development of a false self different from the real self (**theory of multiple self organizations**).
- **Transitional Object:** an object invested with some special meaning usually given to an important person such as the mother, but which is under the child's control, eg a teddy bear or blanket.
- **Good Enough Mother:** a mother who adequately fulfils her caring role but who allows for a gradual disillusionment, thus helping a child develop independence
- **'Holding'** proposed by Winnicott has been modified and adapted for psychotherapy. While administering psychotherapy, the affective and cognitive dispositions of a therapist play important part. This must be differentiated from the cognitive capacity of the therapist to maintain objectivity and focus on selected facts during a discourse – the latter is called **'containing'** (Bion). The affective disposition of the therapist, which helps in restraining oneself from retaliating to negative transferences, is called **'holding'**.

★ **Fairbairn:**

Proposed libidinal, antilibidinal and ideal parts of an object; also extended as libidinal, antilibidinal and ideal self.

Important psychodynamic concepts

Psychic determinism: Psychoanalytic theories uphold psychic determinism i.e. unconscious events play causal role in later experiences. They emphasize developmental psychopathology as the source of adult life difficulties. Psychodynamic therapies emphasize on idiosyncrasy and uniqueness of individuals.

Therapeutic Alliance: This is the ordinarily good relationship that therapist and patient have when working on a common task. Some authors consider that there are three parts to the therapeutic relationship: the therapeutic alliance, the transference and the counter transference. The therapeutic alliance is the rational unwritten implicit contract between doctor and patient. Various clinical problems could occur in the process of psychotherapy and in the establishment of therapeutic alliance. One useful way of considering these issues is suggested by Bateman & Holmes (1995)

Continuity (not immediate threat of termination but may affect progress)	Acting in (enactment within a session)	Acting out (enactment outside the sessions)
Absence	Physical contact	Suicide
Lateness	Persistent questions	Self injury
Breaks	Presents / gifts	Alcohol use
Impasse	Silence	Drug abuse

Interpretation refers to the expression of therapist's understanding of the meaning of feelings, attitudes, defense mechanisms and behaviours currently exhibited during therapy. Interpretation is usually based on psychoanalytical theory practiced by the therapist. Interpretation made by a therapist sheds light on an unconscious process in the patient, therefore making it accessible to the conscious mind.

Transference: The feelings, thoughts and attitudes given to a person in the present (such as the therapist), that do not befit that person but actually originate from a person or figure in the patient's past (such as a parent). E.g., a patient has a very critical father and she perceives her therapist as also being very critical, even though this may not be a feature of the therapist.

- Transference is said to be **bidimensional** – it includes replaying past experiences + seeking new relationship with therapist.
- Transference is unconscious. It is at least partly inappropriate to the present situation.
- Only an aspect of a relationship, not the entire relationship, is transferred
- Transference relationship may not be historically accurate, but is the *current mental representation* of a previously experienced relationship
- May be considered a *communication* of a patient's needs that cannot be verbally expressed

- Transference may include idealizing, erotic or highly negative and denigrating feelings and thoughts.
- Factors increasing transference reactions:
 1. Vulnerable personality, especially people with borderline features – intense and early transference can occur here.
 2. The patient's appraisal of being in a needy and vulnerable or dependent position.
 3. Frequent contact with the therapist or key workers
- Kohut (1971) defined three types of transferences: mirroring transference, idealizing transference, and twinship transference.
 1. *Mirroring transference* is due to significant mirroring failures from the parental figures in childhood. The child feels inadequate and may try to compensate by being perfect or wonderful. In therapy, they are in constant need of a therapist to assure their self-esteem.
 2. *Idealizing transference* is based on the concept that poor self-esteem is not troublesome as long as the individual can be attached to another person with power or prestige. Through the idealization of and identification with external objects, the process of preservation of self-esteem is maintained.
 3. *Twinship transference* evolves when the patient feels comfortable only when the self-object has the same thoughts, values, and appearance. For example, the patient may expect the therapist to feel and act as he or she does.

Countertransference: The therapist's spontaneous feelings and emotions that are evoked when s/he 'tunes in' to the patient's unconscious communication, including the patient's transference. Analysing counter-transference can provide insight into a patient's psychic state in the same sense as analysing transference.

Resistance: The means by which aspects of reality are repeatedly rejected by the patient: s/he refuses to acknowledge them, and they are kept unconscious. Resistance is often noted as a hindrance in the course of therapeutic progress during psychotherapy.

- **Repression resistance** refers to patient's difficulty in gaining access to certain ideas and emotions.
- **Transference resistance** refers to patient's unconscious wish to keep therapeutic relationship similar to past relationships. Negative therapeutic reaction is also a type of resistance.

Termination reactions are temporary setbacks that occur when sessions enter termination phase, due to the dependence of the patient on the therapist.

Negative Therapeutic Reaction: In therapy, this is the process by which a step in the right direction (for example making a new realisation) may be followed by a backwards step (such as

the return of an old symptom, or an episode of acting out). Freud (1923), who first described this reaction, considered it to be secondary to Thanatos and aggressive impulses.

Insight: Being aware of and acknowledging one's mental processes, including ego defence mechanisms.

Acting out refers to performing an action to express unconscious emotional conflicts. While acting out, the unconscious impulse is discharged by means of an action instead of verbalization. These discharges are responses to the '**return of the repressed**' i.e. unacceptable impulses that undergo repression release themselves by acting out. Acting out often occurs in psychotherapy sessions; extreme forms of acting out may be a contraindication for continued therapy.

Repetition compulsion: The concept of the "repetition compulsion" refers to psychological phenomenon in which a person repeats a traumatic event or its circumstances over and over again. The compulsion to repeat is curious because what is repeated is not pleasurable but painful and destructive feeling and action. Freud proposed that repetition compulsion occurs during Id vs. Superego conflicts where Id overrides the superego and presents itself.

'Working through': 'Working through' is a process of unlearning prior misconceptions and learning new constructions. For example, working through defence mechanisms is a process whereby defence mechanisms are observed and recognised in the work of the therapy, allowing the patient to gain insight into them and unlearn them where needed, and potentially use them less destructively. It is a major therapeutic element in long-term therapy though not as often used in short term therapies. 'Working through' requires consistent and insightful therapist guiding the patient.

Franz Alexander suggested that a further step of **corrective emotional experience** was necessary in the process of working through to achieve longer lasting relief. Alexander saw corrective emotional experience as the central part of change secondary to psychotherapy. Processes that take place in a therapy setting give the patient an opportunity to reflect on their past experiences and make necessary behavioural or cognitive and emotional changes to reduce one's difficulties.

Regression in psychotherapy: During psychotherapy, an activation of parts of the patient's personality that is normally hidden may occur. The process by which access to these hidden aspects of personality occur during psychotherapy is called regression. Regression is considered as crucial to successful psychoanalysis. Fear of regression is an important source of resistance to long-term psychotherapy especially in patients with a history of psychosis.

Defence mechanisms: Anxiety about an internal conflict over a wish or impulse, can trigger the use of a defence mechanism. Defence mechanisms are normal- they help us manage the interface between unconscious wishes/impulses and external reality (which include prohibitions over the acting out of wishes or impulses). However, when defence mechanisms are used too rigidly, then they can become problematic because they restrict the flow and spontaneity of human relationships and interactions.

Defence Mechanisms can be divided into 3 main groups (Vaillant, 1977). *(These are described in much greater detail in SPMM SmartRevise Paper A notes. A brief summary is given below to aid Paper B revision).*

1. Immature Defence Mechanisms: These are called immature because they are frequently employed in infancy/early childhood.

- **Acting Out:** the unconscious wish or impulse is expressed and does not remain repressed. E.g., A man has an unconscious wish to have a sexual relationship with someone, and he acts on it
- **Regression:** reverting back to an earlier stage of development. E.g., Feeling unwell and reverting back to a child-like state of being taken care of by a loved one
- **Denial:** the explicit refusal to acknowledge a threatening reality

Kleinian (or Psychotic) Defence Mechanisms: Also considered **immature**, the following were described by Melanie Klein

- **Splitting:** good and bad aspects are split and kept apart. E.g., A patient with Borderline Personality Disorder states that some members of the community team are 'wonderful', 'like angels', and understand her 'perfectly', whilst others are 'awful', 'useless' and 'cruel'. *DDP: Distortion, Denial*
- **Idealisation and Denigration:** when splitting occurs, one side tends to be idealised (seen as perfect) and the other denigrated (seen as all rubbish).
- **Projection:** an unwanted aspect of the self is located in the other. E.g., I don't want to know about my own tendency to be aggressive, so I see someone else as having aggressive qualities. The threat becomes easier to handle if it's external.
- **Projective Identification:** the projection is received and taken in by the person to whom it was directed, and they begin to act as if it was their own quality in the first place. E.g., a woman projecting her untidiness into her husband and accusing him of being messy may result with her husband (normally a tidy person) identifying with it and begin to be untidy. He has become identified with the projection.

2. Neurotic Defence Mechanisms

- **Repression:** unwanted aspects of internal reality are kept out of consciousness, but the patient may still experience some emotions associated with the repressed memory or idea. E.g., a patient may not have a memory of childhood sexual abuse, this may have been repressed, but s/he may experience uncomfortable emotions in the arena of sexual life
- **Intellectualisation:** there's a focus on abstract, theoretical concepts and a distancing from the emotions. E.g., a patient in therapy may protect themselves against painful feelings by always linking their experiences to theoretical concepts.
- **Rationalisation:** a justification is made to explain away some thought or feeling which would rather be kept out of awareness. E.g., In Oesop's a fable a fox wants to eat some

grapes, but they are too far out of reach, and so the fox says to himself 'oh well, they were sour anyway': better to think the grapes were sour and therefore unwanted, than to acknowledge that the fox wanted them and couldn't have them!

- **Reaction Formation:** the feelings/thoughts expressed are the opposite of what is really being thought or felt, but this has to be kept out of conscious awareness as it might be too difficult to handle. For example someone may be very accommodating and pleasing to guests whom one actually finds very difficult, but it's too hard to acknowledge that one doesn't like them and doesn't want them in one's house.
- **Undoing and magical thinking:** this is employed in OCD, where a patient may believe that by doing a certain action (e.g., tapping the wall) a tragedy might be prevented.
- **Displacement:** instead of directing thoughts and feelings towards one person, they are directed somewhere less threatening. For example you may have a wish to express anger towards a partner, but you express it towards the cat instead.

3. Mature Defence Mechanisms

- **Humour:** difficult or unpleasant experiences are made humorous and therefore more bearable
- **Altruism:** attending to others' needs before one's own, this may happen when there is a conflict about attending to one's own needs.
- **Sublimation:** energy from an unacceptable impulse is directed in a socially-accepted way. For example a wish to cut people up may be transformed into a brilliant surgical career.

Practice of psychodynamic therapies

Indications for brief, long-term and supportive psychotherapy

- ★ Indications for **brief psychotherapy**: when the problem the patient presents with a fairly well demarcated, in the context of a difficulty of relative short duration, and when other aspects of the patient's life are functioning reasonably well.
- ★ Indications for **long-term psychotherapy**: suitable for more complex difficulties, that are long-standing, that affect multiple aspects of a person's functioning and usually involve the person's character or personality
- ★ Indications for **supportive psychotherapy**: helpful for periods of transition and adaptation, when a deeper working through particular problems is not required

Contraindications for brief or long-term psychodynamic therapies

- ★ Poor impulse control
- ★ Poor frustration tolerance
- ★ Low motivation.
- ★ Antisocial personality disorder
- ★ Absence of psychological mindedness (ability to scrutinize and verbalize one's own cognitive processes)
- ★ Being in the midst of a major life crisis.

- ★ Poor ego strength (capacity to shuffle oneself appropriately between two different ego states e.g., being a passive and dependent patient vs. being autonomous and plan one's routine life outside the therapy)
- ★ Severe active psychosis
- ★ Poor ability to form and sustain relationships

Brief psychodynamic psychotherapy

- ★ In 1946, Franz Alexander and Thomas French identified the basic characteristics of brief psychodynamic psychotherapy. Since then, many workers such as David Malan in England, Peter Sifneos in the United States, and Habib Davanloo in Canada have influenced the field.
- ★ Time-limited treatment based on psychoanalysis and psychodynamic theory.
- ★ Somewhat more focused on the here and now- the patient's current experience of the world than long term therapy
- ★ Therapists identify and interpret the transference early in the treatment. Therapists formulate a circumscribed focus and set a termination date in advance, and patients work through grief and anger about termination.
- ★ The methods employed in brief psychodynamic therapy include:
 - Goal setting and explicit identification of the anxiety and defenses to be tackled.
 - Focus choosing: Identification of currently active problem (here and now – core conflictual relationship themes that represent cyclical maladaptive patterns are focused). Explore symptom precipitants and associated early trauma and avoidance.
 - Active interpretation: Therapist may guide therapy by use of interpretation at an earlier point than in more prolonged methods.
 - Creating heightened emotional contexts conducive to change (Sifneos & Davanloo)
- ★ **Factors predicting good outcomes, in brief, psychodynamic therapy:**
 - Circumscribed problem
 - Strong motivation
 - Able to express feeling at assessment
 - Psychological-mindedness
 - At least one good relationship
 - Evidence of achievement
 - Not actively suicidal, chronically obsessional or phobic
 - Not grossly destructive or self-destructive; not actively abusing illicit drugs

Supportive Psychotherapy

Includes counseling and supportive techniques used when offering psychiatric follow-up. It aims to offer practical and emotional support, provides an opportunity for ventilation of emotions and guided problem solving discussions.

The primary aim of **supportive psychotherapy** is to support reality testing, provide ego support and attempt to reestablish usual level of functioning.

Usually employed in otherwise healthy patients with overwhelming ongoing crises and those with ego deficits. Also useful in those who are not psychologically motivated to 'explore' themselves. This is not time limited and the therapist must be predictable available in times of need. Problem solving, advice, reinforcement and reassurance are the main tools.

Exploratory or analytical psychotherapy	Supportive psychotherapy
Aimed at exploring the cause for symptoms	Aims at identifying and supporting existing coping skills
Often time limited	No set boundaries – as and when needed
Sessions focused on childhood trauma and developmental difficulties	Focus is on the current life crisis and the support needed
Suitable for patients with reasonable frustration tolerance, psychological-mindedness, intact reality-testing (i.e. no psychosis). Patients must be able to form meaningful object relations.	Can be used even when there is poor frustration tolerance, low degree of psychological-mindedness, impaired reality-testing (i.e. presence of psychosis). Patients could have severely impaired ability to form object relations.

2. Family Therapy

Influence of General Systems Theory:

- Key Figure: Ludwig von **Bertalanffy** (biologist)
- Key ideas: A system is a set of interconnected components that form a whole; The components show properties of the whole, rather than of individual components; Cycles of feedback between different components within the system continuously create and re-create a basis for interaction.

Models of Family Therapy

Dynamic

- Theme: To bring to light forces at play that influence the way a family functions. Emphasizes individual maturation in the context of the family system.
- Theory: There are unconscious processes which, when noticed and worked through, can bring relief to the family's conflictual experiences
- Activity: Makes interpretations, noticing the formation of alliances, dyads and triads between members. Therapists seek to establish an intimate bond with each family member. **Family sculpting** refers to family members physically arranging themselves in a scene depicting individual view of relationships.

Structural (Minuchin)

- Theme: Challenges the patterns of behaviours or interactions that disrupt a family structure.
- Theory: A well-functioning family has a structure: clear hierarchies, boundaries between generations, and well-defined rules. When these are disrupted, problems occur.
- Activity: The therapist challenges the interactions between the generations. Both individual and family sessions used.

Family Systems Approach (Bowen)

- Theme: Emphasizes one's ability to retain individual self in the face of familial tension.
- Theory: An *emotional triangle* is a three-party system where closeness of two members (in either positive or negative sense) tends to exclude a third. This hot triangle leads to symptom formation
- Activity: The degree of enmeshment is analysed. The therapist maintains minimal emotional contact with family members. Bowen also found a tool to analyse history of families across generations – called the **genogram**.

Strategic (Haley)

- Theme: Aims to find the positives in a system and builds on them
- Theory: Problems within families can be maintained by over-emphasising them, so that they end up being maintained rather than resolved
- Activity: Positive reframing: finding the positive in negatively-labelled interactions
- Utilizes the **domino effect**: if one problem is properly addressed, it leads to reduction or resolution of other problems

Psychoeducational

- Theme: The objective is to enhance family support and reduce stress
- Theory: There is a risk of relapse when family interactions are overinvolved, emotionally charged and critical. The course of mental illness, such as schizophrenia, will be affected by these stress levels and the counterbalanced by support available.
- Activity: Focuses on helping families to understand factors that affect stress levels, helps facilitate communication and encourages problem-solving strategies.

Behavioural

- Theme: The aim is to closely observe and evaluate behaviours in the family so as to identify problems and make specific interventions.
- Theory: Behaviour is essentially maintained in a more or less linear model. Symptoms are viewed as learned responses that reinforce dysfunctional patterns of relating.
- Activity: Treatment is symptom-focussed and time-limited. The therapist's personality is not important, but therapist action is.

The **Milan systemic approach** (Palazzoli) gives great emphasis on circular and reflexive questioning. In a circular fashion each family member is asked to comment and reflect on each other's response.

Paradoxical therapy (Gregory Bateson): Therapist makes the patient intentionally engage in the unwanted behavior (called the **paradoxical injunction**) e.g., avoid a phobic object or perform a compulsive ritual. This counterintuitive approach can provide new insights for some patients.

3. Cognitive Behavioural Therapy

Concepts of conditioning

- ★ Operant Conditioning (Skinner) refers to changing behaviour by the use of reinforcement (may be positive or negative).
- ★ Aversive conditioning: punishment reduces the frequency of the target behaviour (e.g., the cat is sprayed with water when it scratches the sofa)
- ★ Reinforcement: a reward increases the frequency of the behaviour (e.g., a dog is given a treat when it fetches the ball)
- ★ In humans, you can use covert reinforcement: the reinforcer is the imagination of something pleasant. The opposite is covert sensitization when an unpleasant thing is imagined.
- ★ Shaping (or successive approximation): a desirable behaviour pattern is learned by the successive reinforcement of behaviours that get progressively closer to the desired one. For example: you want a dog to jump through a hoop of fire, so you reward him walking past the hoop, then reward him when he jumps through the hoop, then reward him when he jumps through the hoop when it's on fire.
- ★ Chaining: reinforcing individual responses occurring in a sequence. For example: in forward chaining a complex sequence such as potty training is broken down into segments and each segment (going to the toilet, pulling one's trousers down, wiping) is rewarded; in backward chaining the segments of behaviour are rewarded from finish to start, for example a child may start with a complete puzzle and then rewards are given for each stage of the puzzle being taken apart.
- ★ Incubation: an emotional response increases in strength if brief, repeated exposure to the stimulus is present. For example air raid sirens may ring for brief periods at intervals, and in-between the anxiety increases. **Rumination** can serve to increase anxiety via incubation.
- ★ Learned helplessness (Seligman): when escape from an aversive stimulus is impossible, trying to escape stops. This was originally a behavioural model for depression.
- ★ Stimulus control or cue-exposure control: This refers to the control a discriminatory stimulus has on the probability of a behaviour (operant response) because of the reinforcement experienced in the past. In therapy, stimulus control refers to controlling the stimuli that lead on to problem behaviour e.g., avoid walking near a pub to control the cue for drinking.
- ★ Habituation is a nonassociative learning in which repeated stimulation leads to reduction in response over time as the organism 'learns' the stimulus. Sensitization is an increase in response to a stimulus as a function of repeated presentations of that stimulus. It is the

opposite in result of habituation – yet the conditions that produce them both are, on the surface at least, the same: repetitions of exposure to the eliciting stimulus.

Therapies based on conditioning

- ★ **Systematic Desensitization (Wolpe):** This is based on the behavioral principle of **counterconditioning** (i.e. gradual approach of feared situation in a psychophysiological state that inhibits anxiety leads to reduction of anxiety response.) and **reciprocal inhibition** (i.e. when anxiety and a relaxed state are co-existent, then anxiety reduces).
 - Systematic desensitization consists of three steps:
 - Relaxation Training
 - Constructing a Hierarchy of Anxieties
 - Desensitization of the stimulus
 - The patient is exposed to a graded hierarchy of anxiety-provoking situations in stepwise fashion. Example: a young woman with a spider phobia is first taught relaxation techniques (such as visualisation), then is exposed to experiences starting from the lowest level of her hierarchy of anxieties (such as going to the cellar where there is the potential to come across a spider), going up the ladder to the highest (for example handling a spider).
 - Several concepts related to systematic desensitization are employed in behavioural therapies.
 - Relaxation produces physiological effects opposite to those of anxiety. In **progressive relaxation (Jacobson)** patients relax muscle groups in a fixed order starting from small muscle groups working upwards.
 - In **mental imagery**, patients are asked to imagine themselves in a place associated with pleasant relaxed memories.
 - In **graded exposure therapy**, relaxation training is not involved, and treatment is carried out in a real-life context though in a hierarchical fashion.
 - **Autogenic training**: a method of self-suggestion whereby the subject directs his/her attention to specific bodily areas whilst carrying out a relaxation exercise
 - **Applied tension**: a technique that is the opposite of relaxation is used to counteract the fainting response (for example in injection phobias)
- ★ **Flooding:** In flooding based therapy, real life (in vivo) exposure happens without any hierarchy: the anxiety is not avoided but tackled head-on! Escaping from an anxiety-provoking experience, in fact, reinforces the anxiety through avoidance conditioning; in flooding this conditioning is targeted.
 - The success of flooding depends on exposing patients for a reasonable duration until mastery and calm composure are gained. Premature withdrawal will reinforce the avoidance.

- In **implosion** (in vitro) or **imaginal flooding**, the phobic situation is confronted through imagination, not in real life.
- Flooding is contraindicated in those with poor stress tolerance or cardiac morbidity that may cause ischemia.
- ★ **Massed Negative Practice**: frequently used in tic disorder, when the patient is asked to deliberately perform the tic movement for specified periods of time, interspersed with brief periods of rest.
- ★ **Habit Reversal Training**: useful for OCD and tic disorders, consisting of:
 - *Awareness training*: becoming aware of what stimuli/situations provoke the behaviour
 - *Competing response training*: teaching responses that counteract the behaviour (e.g., in forearm flexion, the patient practices forearm extension)
 - *Contingency management*: positive reinforcement for the desirable behaviour
 - *Relaxation training*
 - *Generalisation training*: once one component has been mastered; this is generalised to other problem behaviours.
- ★ **Modelling**: In participant modeling, patients learn a new behavior by imitation, primarily by observation, without having to perform the behavior until they feel ready. This is useful when treating phobic children. A variant of the procedure is called **behaviour rehearsal**; wherein real-life problems are acted out under a therapist's observation or direction.
- ★ **Biofeedback**: Involuntary autonomic nervous system can be conditioned by the use of appropriate feedback – this is the principle behind **biofeedback** therapy (Miller). Autonomic functions conditioned include skin temperature, electrical conductivity, muscle tension, blood pressure, respiratory rate and heart rate. It is claimed that various psychosomatic conditions including migraines, asthma, hypertension and angina can be addressed using this technique.
- ★ **Social skills training (SST)**: SST employs a multitude of learning principles to aid in recovery and rehabilitation of long-term serious mental illnesses such as schizophrenia. Following the framework described by Bellack and Mueser (primarily employed in rehabilitation of patients with schizophrenia), there are three forms of social skills training:
 1. *The basic model*: here complex social repertoires are broken down into simpler steps, subjected to corrective learning, practiced through role playing and applied in natural settings.
 2. *The social problem-solving model*: This focuses on improving impairments in information processing that are assumed to be the cause of social skills deficits. The model targets domains needing changes including medication and symptom management, recreation, basic conversation, and self-care.

3. *The cognitive remediation model*: Here the corrective learning process begins by targeting more fundamental cognitive impairments, like attention or planning. The assumption is that if the underlying cognitive impairment can be improved, this learning will be transferred to support more complex cognitive processes, and the traditional social skills models can be better learned and generalized in the community.

Behavioural analysis

- ★ Each behaviour serves a purpose for a person. Identifying such function (may be positive or negative reinforcement) is important to manipulate behaviour through therapy. This forms the principle of functional assessment. Hence, if someone presents with a simple phobia, for example, taking a detailed history to assess the behavioural components must be the first step before prescribing any treatment.
- ★ In practice, behavioural or functional analysis consists of:
 - 1) Identifying Motivating Operations (why is it happening)
 - 2) Identifying Antecedents/Triggers for the behaviour (what triggers it to happen)
 - 3) Identifying the Behaviour that has been operationalized (what exactly happens)
 - 4) Identifying the Consequences of the behaviour, which reinforces it (what keeps it happening)
- ★ This is also termed as the **antecedents-behaviour-consequence** approach of functional assessment. The next step is an active functional analysis, where antecedents and consequences are manipulated in a therapy setting to find their separate effects on the behaviour of interest.
- ★ **Behavioural Treatment plans**: Conducting a Functional Analysis can assist in making a behavioural treatment plan. Identify clearly the problems/symptoms, set short-term and long-term goals and objectives, define specific interventions/actions, and decide how outcomes will be measured (e.g., Use of a chart to mark symptom reduction, or to measure change in incidences of aggressive behaviour).
- ★ **Measuring outcomes of behavioural interventions**: When measuring a specific behaviour, various dimensions of behaviour can be used as quantifiable measures of the behaviour. These dimensions are:
 - **Repeatability** refers to the frequency of the behaviour.
 - **Temporal extent** refers to the duration of each instance of behaviour.
 - **Temporal locus** refers to the time point at which each instance of behaviour occurs.
 - **Response latency** is the measured time interval (reaction time) between the onset of a stimulus and the initiation of the response.
 - **Inter-response time** is the amount of time between two consecutive responses.

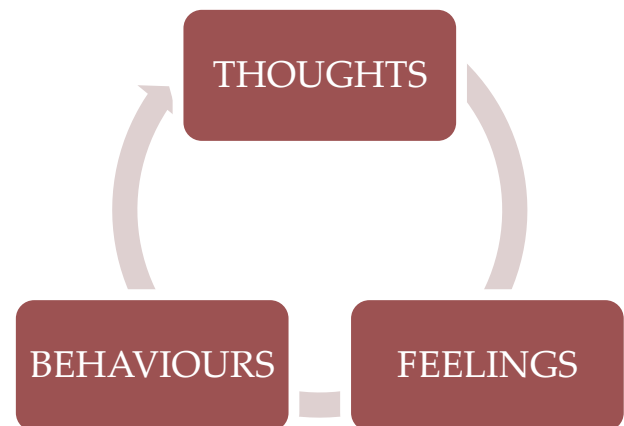
Cognitive Therapy

The Cognitive Model for non-psychotic disorders

Aaron Beck is the major proponent of cognitive therapy (CT). The cognitive model proposes that one's view – or cognitions about what happens to them – determines affective and physical changes and other associated psychopathology. In fact, a triangular relationship exists between **thought-feelings-action**.

Cognitive Dysfunctions: CT identifies three levels of cognitive dysfunctions: negative automatic thoughts, conditional assumptions and basic or **core beliefs** (or schemas).

- **Negative automatic thoughts** or **cognitive distortions** (cognitions that automatically arise in certain situations or as a reflex response to certain behaviours e.g., I have to change jobs → 'I won't be able to cope with it')
- **Conditional** assumptions (rules or guidelines for life – they usually start with the phrase 'I must' or 'I should' e.g., 'I **should** always be pleasant')
- **Core beliefs/schemas** (one's appraisal of oneself and they usually begin with – 'I am' e.g., 'I am no good').



Maladaptive Cognitive Assumptions

A mnemonic that may come handy is **MOSPAD-C**.

Distortions	Explanation and example
Minimisation and magnification	E.g., You pass an exam → 'but I only just scraped through it' (Minimisation of positive outcome) Or You fail an exam by one mark → 'I totally flunked it' (magnification of negative outcome)
Over-generalizing	E.g., A teacher sees one student sleeping in her class and assumes the whole class is bored
Selective abstraction	E.g., You fail one exam and focus on this, rather than considering all the exams you did pass
Personalisation	E.g., In a work re-configuration, the whole team is made redundant, and you think 'it happened because of me.'
Arbitrary Inference	E.g., 'I did my CASC on the last day of autumn last year and failed, the same will happen again this year.'
Dichotomous Thinking (Black and white thinking)	E.g., 'Either I will pass the exam and my life will be a success or I will fail the exam, and my life will be a complete failure.'
Catastrophization	E.g., I have palpitations, and I think 'this is a heart attack, I will die!'

Cognitive assumptions in psychiatric disorders

Disorder	Cognitive assumptions
Depression	Negative view of self/past (worthlessness), world/present (helplessness), and future (hopelessness)
Panic disorder	Catastrophic misinterpretation of physiological experiences
Paranoid personality disorder	Negative global external attribution bias
Obsessive-compulsive disorder	1. Thoughts are as powerful as actions (<i>thought omnipotence</i>) 2. Alternative or substitute action can undo or compensate for another thought or action.
Suicidal behaviour	Hopelessness and cognitive constriction (one-way exit)
PTSD	Guilt and self-blame, feelings of loss of control.

Maintaining factors: The presence of cognitive distortions alone is often insufficient to explain the maintenance of several symptoms, especially anxiety in anxiety disorders. If dysfunctional cognition is the source of anxiety, how is anxiety maintained?

Anxiety is maintained by

1. Situational **avoidance**/escape behaviour, which strongly reinforces the anxiety response.
2. **In-situation safety behaviours** (Salkovskis): Variety of subtle behaviours/internal mental processes that most patients engage in while in a fearful situation. These are actually intended to prevent feared outcome. E.g., bowing the head down and gently leaning leftwards when having a panic, with a hope to increase heart's circulation. This makes one believe erroneously that this behaviour is the reason why one survived the 'attack'. Commonly, patients engage in a large number of different safety behaviours at any time during a crisis.
3. **Attentional deployment:** Patients with panic or hypochondriasis fear certain bodily sensations, catastrophically elaborating them. As a result, they selectively pay more attention to such body parts, becoming aware of benign sensations that others do not even notice.

DYSFUNCTIONS CAUSED BY SAFETY BEHAVIOURS

Generating new symptoms

e.g., Hyperventilation, a 'safety behaviour' in response to feelings of choking during panic attacks, produces physiological acid base changes that leads to symptoms such as paresthesias, dizziness etc.

Worsening existing symptoms

e.g., active thought suppression, a safety behaviour seen in PTSD and OCD, increases the probability that the intrusion/obsession will occur.

Escalating undesirable social responses

e.g., those with social phobia who attempt to cough whenever they blush in order to camouflage it may actually attract more response by coughing.

Maintaining existing symptoms

4. **Rumination:** This serves to increase the subjective likelihood of a negative event which is ruminated upon. Rumination is not a problem-solving tool in most of those with depressive/anxious cognitive style – instead it serves to elaborate or make threats more abstract and hence difficult to cope with.

Techniques employed in CBT

- ★ **Guided discovery** refers to a style of the interview where sensitive questioning allows patients to reach new interpretations/ reframe their cognitions independently; therapist guides self-discovery and does not prescribe the solution.

Stages of guided discovery	
Stage 1: Asking informal questions	Delineate patient's concerns
Stage 2: Listening	To be clear about exact issues
Stage 3: Summarising	To demonstrate understanding and to revise
Stage 4: Synthesizing / analytical questions	'How does all the information discussed fit with your idea that you are a failure?'

- ★ **Questioning identified beliefs:** "What evidence do I have for this belief?" "What alternative explanations could there be?" and "What are the advantages and disadvantages of thinking in this way?"
- ★ **Testing Predictions.** Predictions about specific symptoms indicating imminent catastrophe can be tested in sessions. Simple procedures can be used to bring on feared symptoms. If the exact or similar sensations to those involved in the patient's concerns can be reproduced, it helps to disconfirm a catastrophic interpretation and thus build up belief in the alternative explanation.

CBT approach to specific disorders

CBT for anxiety

- ★ **Behavioural experiments** follow construction of hypothesis about symptoms e.g., a hypothesis that 'when one has a panic attack he will not get suffocated even if he is not hyperventilating or holding tight to his chest' is tested through homework by the patient conducting an experiment of not holding tight and not hyperventilating and reporting the event in next session. This helps to:
 1. Establish that a feared catastrophe will not happen;
 2. Discover the importance of maintaining factors;
 3. Discover the importance of negative thinking;
 4. Find out whether an alternative strategy will be of any value; and
 5. Generate evidence for a non-disease-based explanation
- ★ **Imagery modification:** Visual imagery of threatening stimuli can be modified in those with anxiety.

- ★ **Cognitive restructuring** refers to modifying or ‘reframing’ one’s thinking style to see a different, non-anxiety inducing perspective.
- ★ **Dropping Safety-Seeking Behaviors.** Safety behaviours such as checking, reassurance seeking—maintain health anxiety. Patients can test out the effects of these behaviors for themselves by conducting an alternating treatment experiment. This experiment involves, first, increasing the target behavior for a day—such as bodily checking and information seeking—and, second, monitoring anxiety, bodily symptoms, and strength of belief at regular intervals. On the next day, the patient has to ban completely carrying out the target behaviour, but once again, anxiety, symptoms, and strength of belief are monitored at intervals. The resulting data is reviewed and graphed at the next session.

CBT for OCD

- ★ **Thought stopping** is a behavioural technique in OCD. Here the patient shouts ‘stop’ or applies an aversive stimulus (such as pressing his nails) to counteract the obsessional preoccupation.
- ★ **Thought postponement** (OCD) refers to postponing the thought until a specified time and not to delaying it until then to gain control.
- ★ **Exposure and response prevention for OCD** refers to a paradigm similar to systematic desensitization where the hierarchy of obsession provoking situations is created and exposed to while preventing any compulsions or responses being carried out.

CBT for hypochondriasis / health anxiety:

- ★ **Self-monitoring** by using health-anxious thoughts diary.
- ★ An **inverted pyramid technique** is helpful for addressing overperception of risk. The patient is asked to estimate the current number of people with a particular symptom (i.e., those who have it today), the number for whom it persists, the number who consult their doctors, the number who are told they need tests, the number who are told the problem is serious, and the number who are not successfully treated.
- ★ **Selective Physical Attention Experiments:** Patients are asked to focus on a specific body part for several minutes (one that is not a current cause for health anxiety); after which, they are asked to describe any bodily sensations they notice. Most patients will detect sensations that they were unaware of before the experiment—for example, tightness in throat, tingling in feet. This exercise is helpful as a demonstration of the effects of symptom monitoring and bodily checking.

CBT approach to psychosis

The CBT approach to psychotic symptoms is based on two different models:

1. **Stress-vulnerability model of schizophrenia.** Focuses primarily on stressors capable of triggering or exacerbating symptoms. Helps a psychotic individual deal with these stressors and triggers.
 - a. Coping Strategy Enhancement is the primary tool for treatment. These strategies are conventionally divided into affective strategies (e.g., relaxation, sleep, etc.), behavioural strategies (being active, drinking alcohol, etc.), and cognitive strategies (distraction, challenging voices, switching attention away from voices, etc.).
 - b. It is assumed that certain strategies are unhelpful and generate stress in the individual. Relapse indicator identification and control is another strategy used.
 - c. This model primarily aims at relapse prevention and functional recovery.
2. **Continuum model:** Here the emphasis is on the similarity between normal (but strongly held) beliefs and delusional beliefs.
 - a. This model primarily aims at symptom relief – especially delusions.
 - b. This approach encourages the individual to weigh evidence that contradicts a delusion.

Though CBT is gaining popularity in treating psychotic symptoms often, clinicians are unsure about the symptoms targeted in CBT. According to Birchwood, the target is the emotional dysfunction that accompanies psychotic experience and not the psychotic symptoms per se.

Turkington described the following elements in CBT for psychosis:

- Therapeutic alliance – not colluding with delusions but validation.
- Improving medication adherence.
- Providing alternate explanations to unusual experiences e.g., normalisation.
- Decreasing the impact of positive symptoms e.g., addressing the omnipotence of voice.
- Graded reality testing using peripheral questioning and inference chaining.

CBT complements the recovery model for schizophrenia. The interpretation of existing evidence for CBT in psychosis is highly controversial; while bodies such as NICE and British Psychological Society have embraced CBT in their framework of treating psychosis, many experts question the validity of the appraised evidence. It is still unclear when and how CBT should be delivered, what are the most effective and essential components of such CBT, reliability of CBT among various therapists, what kind of patient is the most suited. (e.g., see Taylor & Perera, 2015)

4. Group Therapy

Development of Group Therapy

Group methods were developed in the early 20th century following observations of beneficial group effects in tuberculous patients. JH Pratt was the first major proponent to observe the beneficial effect of the group when he ran general-care instruction classes for recently discharged TB patients. In the 1920s, it was developed by T Burrow, and then furthered after WWII, when a large number of soldiers required psychological treatment.

Group Processes: The central premise is that the behaviour and dynamics of the whole (the group) cannot be derived solely from its constituting parts (the individuals within the group). Once formed, the group will develop its own way of existing/it's own culture, with particular norms, roles, relations and goals.

Group alliance refers to the quality of the relationship that develops between each individual member and the therapist. **Group cohesion** refers to the sense that the group is working together towards a common goal. **Group coherence** is a more evolved group state where the group goes beyond cohesion and becomes self-evolving and able to work through conflicts. **Positive identification** refers to an unconscious group mechanism in which a person incorporates the characteristics and the qualities of the group. **Catharsis** refers to the process by which mere expression of ideas and conflicts is accompanied by an emotional response which produces a sense of relief.

Types of group therapies

According to the objectives and degree of leadership:

1. **Highly specific target oriented** groups include structured groups for drug use or alcohol use, activity groups like occupational therapy groups, etc. These groups have a **high level of leader input**.
2. Psychodrama, music therapy, systems-centred groups are some **less specific** therapies but are **highly directed** by the leader or therapist.
3. Problem-solving therapy and psychoeducational groups are **highly specific** but have a **low level of therapist activity**.
4. Support groups, art therapy, interpersonal therapy and groups like Tavistock model analytic groups have a **low level of leader activity** and have **low specificity** with respect to treatment goals.

According to the membership:

1. **Homogeneous** groups include members who are comparable in age, diagnosis, background, etc.
2. **Heterogenous** groups include people of varying categories.
3. **Closed groups** have a fixed number and composition of patients. If any group member leaves, no new members are included.
4. In **open groups** no fixed limit exists for number of members; membership is more democratic, and new members can come in whenever someone leaves.

According to the mode of therapy:

1. **Activity groups**- used for patients who are unsuitable for other group activities. Focuses may be art, computing and gardening. It is mainly used in LD, chronic psychosis, and other disorders with functional impairment.
2. **Support groups**-peer support in LD, chronic illness and also for those caring for others.
3. **Problem-focused groups**-alcohol dependence, drug dependency, sexual deviancy
4. **Psychodynamic groups**-Aim of lasting change through exploratory therapy
5. **Behavioural groups** – e.g., for phobia therapy

Psychoanalytic/dynamic group therapy

Analytic/dynamic groups include an examination of the conscious and unconscious processes in the group, including resistance, transference, counter-transference.

- ★ **Bion:** Described that when a group gets derailed from its task, it goes into one of three basic states:
 - **Dependency** (group members become dependent on one another and try to elicit protection)
 - **Pairing** (it is hoped that the formation of a partnership in the group might bring forth a new resolution)
 - **Fight-flight** (an attack or withdrawal mode)

The above 3 features are called **basic assumptions**. A 4th basic assumption was introduced by Hopper. This is called **massification/aggregation** where a rigid fusion of identities leads to loss of individuality, or extensive withdrawal leads to loss of mutual dependence.

- ★ **Foulkes:** He described the group 'matrix': a web of communications and relationships belonging to a particular group.
 - *Foundation matrix:* commonalities existing even between total strangers, attributable to characteristics of the human species (e.g., language, race). Foundation matrix is a precondition for the later evolving dynamic or group matrix.

- *Dynamic matrix*: the ever moving and ever developing exchanges that happen between group members on the basis of getting to know one another

Factors influencing communication in a group matrix: (Foulkes, 1964)

1. Mirroring
2. Exchange
3. Free floating discussion
4. Resonance
5. Translation

The above mostly applies to a psychodynamic group setting.

★ **Yalom's curative factors:**

Yalom cited 11 'curative' factors responsible for the change in groups. The curative factors include the instillation of hope, universality, imparting information (feedback), altruism, corrective recapitulation, socialisation techniques, imitative behaviour, interpersonal learning, group cohesiveness, catharsis and existential factors. Of these, cohesiveness, and learning from feedback are valued positively though other factors may also be important.

Expressive Therapies

- ★ **Psychodrama** (Moreno): the therapeutic dramatization of emotional problems. The therapist or leader is highly active and takes on the role of the **director**. The patient has the role of **protagonist**, with another group member representing someone significant in the patient's life (this role is called the **auxiliary ego**). Other group members act as the **audience**. **Soliloquy** is a monologue-like recital of thoughts and feelings. **Role reversal** refers to the exchange of the patient's role for the role of a significant person. The **double** refers to the auxiliary ego acting as the patient. The **multiple double** refers to several egos acting as the patient. **Mirror technique** refers to an auxiliary ego imitating the patient and speaking in the proxy.
- ★ **Art Therapy**: May be practiced as 'art as therapy' and 'art in therapy'. Traditionally psychoanalytic (with the interpretation made by the therapist based on unconscious processes), now art therapists employ a number of other approaches, such as cognitive or person-centred.
- ★ **Music Therapy**: Similar to art therapy, offering emotional expression through music as facilitated, guided and supported by the therapist. The therapeutic alliance is the most important component.
- ★ **Support Groups**: Usually formed of people who have similar problems, or who have had similar experiences. The aim is to facilitate the sharing of thoughts and feelings, provide sympathetic understanding, sometimes give advice.

Other Group Therapies

- ★ **Cognitive Behavioural Groups:** Important aspects (described by White & Freeman) comprise:
 - Group Cohesiveness: the degree of personal interest of the members for each other
 - Task Focus: goals are defined; tasks may include cognitive restructuring through behavioural experiments
- ★ **Psychoeducational Groups:** These are designed to educate group members about a variety of topics, depending on what the focus of the group may be, for e.g., alcohol misuse or living with phobias. Mental health experts, or peer counsellors, or members of the community may lead them.
- ★ **Skills Groups:** Here the focus is on learning about and developing particular skills, such as budgeting. Members can support the development of one another's progress.
- ★ **Therapeutic Communities:** The four major principles on which a **therapeutic community** is based are exemplified by the Henderson hospital model. According to this model, the major components are (**mnemonic CPD-R**)
 - **Communalism** (Staff are not separated from inmates by uniforms or behaviours, mutual helping and learning occurs)
 - **Permissiveness** (tolerating each other and realising unpredictable behaviour can happen within the community)
 - **Democratisation** (shared decision making and joint running of the unit) and
 - **Reality confrontation** (self-deception or distortions from reality are dealt with honestly and openly by all members without formalities).

5. Other Therapeutic Models

Interpersonal Therapy (Klerman & Weissman)

- ★ Aims to improve interpersonal functioning and may be offered in conjunction with medication. Conducted over 12-16 sessions. Involves giving 'sick role' to the patient.
- ★ Based on the premise that emotional problems are best understood by studying the interpersonal context in which they arise. It is a time-limited, 'here-and-now' focused therapy. Illnesses are viewed as 'medical disorders'. Interpersonal events are not essentially causal – but understanding their role in the illness and resolving the interpersonal problem assists the route to recovery.
- ★ The focuses of treatment are the current interpersonal relationships and their relationship to the development of illness. Inventory of all close relationships is created in early part of therapy. The treatment lasts for 12 to 16, hour-long weekly sessions.
- ★ **Therapist's stance:** In IPT the therapist is an explicit ally and advocate for the patient. The therapist is nonjudgmental, expresses warmth and positive regard for the patient and congratulates the patient as progress in the problem areas is made. The therapist works with the patient and for the patient and believes that the patient's problems can be solved. This does not imply that the therapist accepts all aspects of the patient, as that would preclude any stimulus for change. The therapist always tries to have the patient find the solution for the problems discussed in the session. However, the therapist is not afraid to make suggestions or provide direct advice when they seem useful.
- ★ Areas of focus:
 - Role Transitions (e.g., job change, marriage)
 - Interpersonal disputes (e.g., conflicts at workplace, disputes with a family member)
 - Grief (loss of a loved object/relation)
 - Interpersonal deficits (e.g., unfulfilling relationships, social inadequacy)
- ★ Indicated in mild to moderate depressive illness and bulimia nervosa. © IPT does not focus on bulimic symptoms per se. Instead, a detailed assessment culminating in an "interpersonal inventory" identifies core interpersonal problem(s) that become the focus of treatment. Hence, IPT may be particularly helpful for clients who have become "stuck" in their eating disorder for reasons associated with problematic relationships.
- ★ **Evidence base:** IPT has been shown to have an efficacy that is comparable to imipramine in an NIMH-sponsored trial. At least 2 studies have compared IPT with CBT head to head in patients with depression and comorbid personality disorders. Both concluded that IPT was less effective than CBT in such comorbid presentations. But in general patients without axis 2 disorders respond better to any form of psychotherapy for depression.

Dialectical Behavioural Therapy (Marsha Linehan)

- ★ Developed as a treatment for Borderline Personality Disorder, especially to reduce self-harm. DBT addresses the difficulties faced by a patient with BPD in a hierarchical fashion starting from self-harming behaviours, moving on to therapy interfering behaviours and later behaviours reducing the quality of life.
- ★ The four **modes** of treatment in DBT are as follows: (1) group skills training, (2) individual therapy, (3) phone consultations, and (4) consultation team
- ★ Key Techniques (mnemonic **DICE**)
 - Distress tolerance includes accepting, finding meaning for, and tolerating distress. This includes crisis survival strategies such as distracting, self-soothing, improving the moment, and thinking of pros and cons and acceptance skills such as radical acceptance, turning the mind toward acceptance, and willingness versus willfulness.
 - Interpersonal effectiveness training is very similar to assertiveness and problem-solving training.
 - Core mindfulness training - learning to monitor internal mental states.
 - Emotion regulation skills form an important part of DBT.
- ★ DBT also involves **social skills training** such as meditation, assertiveness training, etc. Another approach commonly employed in DBT is **validation** - recognizing distress and behaviours as legitimate and understandable but ultimately harmful.

Cognitive Analytic Therapy (Anthony Ryle)

- ★ Brings together cognitive and analytic ideas. Can be used in depression, anxiety, personality disorders.
- ★ **Central concepts:** Two theoretical concepts that form the basis are – (PSM) procedural sequence model and role-repertoires.
- ★ **Procedural sequence model** is an attempt to understand aim-directed action. Any aim-directed activity follows ordered sequences of aim generation, environmental evaluation, plan formation, action, evaluation and, procedural revision. Some procedural sequences may be faulty but repeated without revision. These result in repetitive difficulties seen in some psychiatric patients. Certain patterns are described below:
 - **‘Traps’:** seen as negative assumptions that produce consequences, which in turn reinforce assumptions. E.g., ‘I’m not interesting to other people. Therefore I won’t go to social events, therefore I remain out of touch with others, with no-one interested in me’. Depressed-thinking and phobic-avoidance traps are other examples.
 - **‘Dilemmas’:** a person acts as though available actions or roles are limited or polarised. E.g., ‘either I look after her in her distress (she’s the victim) or else she looks after me (I’m the victim)’. Another example is the placation dilemma: A submissive individual → fears consequence of expressing anger → submits repeatedly to others → deep resentment grows → ends in a misplaced outburst of anger → negative consequences occur → strengthens the faulty

assumption and fear of expressing anger. The choices here are narrow – be placid or be explosive, which are false choices.

- **‘Snags’:** appropriate roles or goals are abandoned because others would oppose them, or they are thought to be ‘forbidden’. E.g., I cannot tell my parents I’m scared of getting a promotion because they will think I’m pathetic and not speak to me again
- ★ **Restricted role repertoire:** Undue restriction in the total number / variety of procedural sequences (repertoire) may occur due to the impoverished environment, childhood abuse, etc. Such restricted repertoires lead to neurotic difficulties.
- ★ ***Treatment:*** 16-24 sessions in three phases.
 - Initial phase: an exploration of traps, dilemmas and snags. Therapist writes formulation letter
 - Middle phase: working through problems with the use of diagrams exploring ‘target problem procedures.’
 - Ending phase: both patient and therapist write goodbye letters
- ★ **Applying CAT to Borderline Personality Disorder:** Various levels of difficulties are noted in borderline personality disorder.
 - Normally individuals deploy a wide range of flexible **reciprocal-role templates** as needed during social and interpersonal interactions. Those with borderline personality disorder deploy only a small number of highly maladaptive reciprocal roles. By reciprocal roles, it is meant that when a subject assumes one pole the opponent is pressurized to take up the opposite pole to interact. (Mother – son; teacher-student, etc.).
 - In addition, normal individuals maneuver a smooth transition between roles, for e.g., from being a teacher in the classroom to a colleague in the staffroom. But patient with the borderline disorder show an oversensitive (**‘hair-trigger’**) response to stimuli, resulting in unwarranted changes.
 - Capacity for conscious self-reflection and self-control are also impaired in borderline states.

Transactional Analysis (Eric Berne)

- ★ Examines interactions between people
- ★ Key ideas: there are 3 main ego-states people consistently use:
 - **‘Parent’** e.g., shouting at a colleague when they have made a mistake because this is your own experience of how your parents behaved. This describes a ‘criticising’ parental state, but there may also be a ‘nurturing’ one: taking care of others, as though they were children.
 - **‘Adult’** e.g., making an objective appraisal of reality, behaving in a rational/reasonable way towards others

- **'Child'** e.g., getting into a strop if you are criticised for not doing something correctly.
This is also the source of emotions, spontaneity and creativity

Humanistic psychotherapy

- ★ Originally was promoted as a “third force” in psychotherapy.
- ★ Humanistic therapists believe that each of us has the responsibility for finding meaning in our own lives. Therapy is seen only as a way to help people to make their own life choices and resolve their own dilemmas. To help clients make choices, humanistic therapists strive to increase *emotional awareness*.
- ★ There is a great deal of importance placed on the therapist-client relationship. Most other approaches also recognize the importance of the therapist–client relationship, but they view the relationship primarily as a means of delivering the treatment. But in humanistic therapy, the relationship is the treatment.

A. Client-Centred Therapy (Carl Rogers)

- ★ A central aspect is the notion that “every individual has the motivation and ability to change, and he or she is the best person to decide on the direction of change”. According to Rogers, if clients are successful in experiencing and accepting themselves, they will achieve their own resolution of their difficulties
- ★ Client-centered therapy is **nondirective**. Because of this basic respect for the client’s humanity, client-centered therapists avoid directing the therapeutic process. The client is encouraged to focus on current subjective understanding. The therapist is encouraged to be warm, genuine and to suspend judgement. The patient is believed to have vast resources to understand and help him/herself, and the therapist’s goal is to facilitate this process.
- ★ Deals with the **notion of self-concept**: ‘the organised, consistent set of perceptions and beliefs about oneself’. It emphasizes the importance of ‘therapeutic attributes of genuineness’, ‘unconditional positive regard’ and ‘accurate empathy’.
- ★ **Unconditional positive regard** involves valuing clients for who they are and refraining from judging them.
- ★ The **Q-sort technique** developed from client centered therapy involves a person sorting cards with statements on them into piles.

B. Gestalt Therapy (Perls and Goodman)

- ★ An existential and humanistic psychotherapy focussing the patient’s experience in the present and emphasises personal responsibility.
- ★ Central aspects of the therapy:
 - **Phenomenological method**: aims to increase awareness through repeated observations and inquiry
 - **Dialogical relationship**: therapist attends to his/her own ‘presence’ and creates a space for the client to do likewise. This can be described as ‘inclusion’: supporting the presence of the client (including his/her resistance to being present)

- *Field-theoretical strategies*: this includes a focus on both physical/environmental realities of the client, and those related to the client's mental processes and character structure
- *Experiential Freedom*: a move towards action: trying something new, not just talking about it

Mentalization-Based Therapy (Bateman and Fonagy)

- ★ Derived from attachment theory.
- ★ *Mentalizing* is the capacity to perceive others' and one's own actions and emotional states as meaningful.
- ★ Central concepts include:
 - Maintaining a curious/not knowing stance
 - Understanding of the patient's subjective experience through empathy
 - Validating the patient's experience.
 - The goal is to increase the patient's mentalizing capacities.
- ★ Mentalisation based treatment for Borderline PD started with the psychoanalytically oriented partial hospitalization programme with standard psychiatric care which is now fully evolved as a mentalisation-based treatment that includes individual and group therapy (Bateman & Fonagy, 2004).
- ★ **The key features of MBT:**
 - a. The therapist focuses on patients' current mental state to build up representations of internal states. The therapist avoids situations in which the patient talks of mental states that cannot be linked to subjective reality. Thus, there is a deviation from psychodynamic therapy in the following aspects:
 - i. De-emphasis of hidden unconscious concerns in favour of conscious or near-conscious content
 - ii. Less focus on the past as it is represented in the present;
 - iii. The aim of therapy is not insight but the recovery of mentalization
 - iv. The therapist avoids describing complex mental states (such as conflict, ambivalence, and unconscious) and is asked to make "minor interpretations" referring to ideation that is only slightly beyond the boundaries of the patient's conscious thinking.
 - b. Therapy creates a '**transitional area of relatedness**' - here thoughts and emotions can be trained.
 - c. Any enactments during treatment are not interpreted in terms of unconscious but in terms of the situation and emotions immediately before the enactment.

Eye Movement Desensitization and Reprocessing (Shapiro)

- Premise: When a trauma occurs it seems to get locked in the nervous system with the original picture, sounds, thoughts and feelings. This material can combine fact with fantasy and with 'images' that stand for the actual 'emotions'. The eye movements used in

EMDR 'unlock the nervous system' (desensitise) and allow the brain to correctly process the experience (reprocessing).

- This is based on a highly hypothetical surmise that REM sleep helps in processing the unconscious material and reproducing eye movements that are seen in REM can induce a similar process while awake. This hypothesis has not been proven yet.
- Originally used with Vietnamese war veterans suffering from Post Traumatic Stress Disorder

Transtheoretical Model (Prochaska and DiClemente)

- ★ This was developed largely in response to increasing divergence in the practice of psychotherapy, and the authors attempted a (transtheoretical) synthesis among the various therapeutic systems. They identified five common processes of change from analyzing 18 psychotherapy models. These processes were
 - Consciousness raising: helping the patient gather information about self and the problem
 - Choosing: increasing awareness of healthy alternatives,
 - Catharsis: emotional expression of the problem behaviour and the process of change,
 - Conditional stimuli includes stimulus control and counterconditioning,
 - Stimulus control: Avoidance of stimuli associated with the problem behaviour and the operant extinction cueing effect of the stimulus on behaviour.
 - Counterconditioning: Training an alternative, healthier response to the cue stimuli.
 - Contingency control: Positive reinforcement from others and self-appraisal and improving self-efficacy by self-reinforcement.
- ★ From these five processes of change, Prochaska and DiClemente identified six stages of change. These are (1) precontemplation, (2) contemplation, (3) Preparation, (4) action, (5) maintenance, and (6) relapse.
- ★ In the **precontemplation** stage a person is not even considering changing his or her behaviour, does not see the behaviour as a problem, minimizes and denies associated risks, and avoids information to the contrary. In the **contemplation** stage, the person has become aware of why the behaviour is a problem but is ambivalent about changing, and likely sees equal or more benefits than costs from the behaviour. During **preparation**, the person has made a decision to change, and is planning a strategy for change, but has not yet taken action. In **action**, the person has implemented a plan and is changing the behaviour. In **maintenance**, the person has been able to sustain the change and avoid reverting to problem behaviour for a significant period of time. In successful patients, this usually is the last stage that continues lifelong. In **relapse**, the person does revert to problem behaviour, 'back to square one' – this does not happen to everyone.

- ★ These stages are not linear in sequence but rather cyclical, in that a person can relapse and reenter at a later stage such as preparation. The stages do not operate in an invariant sequence (unlike Piaget's models). Each stage can be moved into back and forth (reversibility). The proposed stages are not qualitatively different.

Motivational Interviewing (Miller & Rollnick)

- ★ It is often used together with TTM and stages of change. In line with Roger's client centred therapy, Miller and Rollnick did extensive work with substance-abusing patients and developed motivational interviewing.
- ★ The major principles are
 - It is more effective to work collaboratively with patients rather than directly challenge them to change their behaviour.
 - Resolving the ambivalence towards changing can increase intrinsic motivation to change – this increase in motivation is the main goal of the motivational interview.
 - A change coming from the patient is more powerful than that prescribed by a therapist.
- ★ While psychotherapies such as CBT assume that a patient is already in an action stage of change and ready for treatment, motivational interviewing uses TTM and evaluates the readiness to change before inducing an action.
- ★ There are five general principles of motivational interviewing: (1) express empathy, (2) develop discrepancy, (3) avoid argumentation, (4) roll with resistance, and (5) support self-efficacy.

Behavioural Couples Therapy

- ★ A specific intervention for alcoholism. It works directly to increase relationship factors conducive to abstinence. It assumes that family members can reward abstinence
- ★ Patient and the spouse are seen together in BCT for 15 to 20 outpatient couple sessions over five to six months. The couple starts BCT soon after the substance user seeks help.
- ★ The therapist arranges a daily "sobriety contract" in which the patient states his or her intent not to drink or use drugs that day (in the tradition of one day at a time), and the spouse expresses support for the patient's efforts to stay abstinent.
- ★ BCT increases positive feelings and constructive communication e.g., "Catch Your Partner Doing Something Nice" is a part of BCT that asks spouses to notice and acknowledge one pleasing behaviour performed by their partner every day.
- ★ Evidence base exists to support BCT.

- In heterosexual couples in which men are entering outpatient treatment for alcoholism or other drug abuse, behavioural couples therapy was more effective than individual-based therapy for improving the psychosocial functioning of their children

Acceptance and Commitment Therapy

- ★ Part of third-wave CBT. Draws upon a basic account of language (Relational Frame Theory)
- ★ Emphasises working on the way people relate to their thinking and feeling, rather than directly trying to challenge or change this
- ★ Theoretical underpinnings:
 - *Cognitive fusion* e.g., I think I am useless, this belief influences my behaviour (I don't do anything), therefore reinforcing my belief
 - *Experiential avoidance* e.g., I feel anxious, and instead of staying with the anxiety, I do everything possible to avoid it
- ★ **Strategies**
 - **Acceptance:** taking a position of non-judgemental awareness towards thoughts, feelings, sensations as they arise
 - **Cognitive Defusion:** the opposite of cognitive fusion e.g., I will try to step back and observe my thoughts
 - **Contact with the present moment:** mindfulness forms the foundation for experiencing the present moment
 - **Self-as-context:** e.g., 'I think I'm useless. Therefore I am useless' - the person's identity is caught up in a particular thought
 - **Values:** patients are encouraged to explore their values: their deeper sense of purpose and meaning; choice and freedom of action are the main focus
 - **Committed action:** learning to move in a valued direction, while in the presence of unwanted or painful thoughts and feelings
- ★ Therapeutic stance: The clinician sidesteps 'literal' language and use metaphors, paradoxes and experiential exercises ; the approach is more like that of a coach, helping the patient get a feel of what is happening, rather than explaining how it works.
- ★ **Evidence base:** Evidence has been accumulating for the past 10 years. Effectiveness has been demonstrated in helping with depression, work stress, psychosis, substance abuse, chronic pain and borderline PD. It has a similar effect to CBT.

Mindfulness-Based Cognitive Therapy

- ★ Developed by Segal and Teasdale for people vulnerable to *repeated episodes of depression*

- ★ Background: even after someone recovers from an episode of depression, even small changes in mood can lead to large-scale negative thoughts and emotions, which can trigger relapse. MBCT addresses this vulnerability.
- ★ **Structure:**
 - 8 weeks of mindfulness classes. Mindfulness, derived from Eastern spiritual practices, encourages participants to change their relationship to thoughts, feelings and body sensations, rather than try and change those thoughts, feelings or bodily sensations. It means noticing and observing with curiosity and compassion.
 - Education classes: learning about depression
 - Exercises derived from cognitive therapy: demonstrating links between thoughts, emotions and bodily sensations
- ★ **Evidence base:** MBCT is associated with 44% reduction in depressive relapse risk. It is included in the 2009 NICE Depression Guideline for people who are currently well, but who have experienced three or more past depressive episodes.

6. Outcome Research in Psychotherapy

Outcome measurement in psychotherapy

- ★ Psychotherapy is made up of non-specific as well as specific factors. Hence it is difficult to confidently ascribe outcome in psychotherapy just on particular features of one therapy, especially as most psychotherapies share many features.
- ★ To best measure specific effects of specific psychotherapy treatments, the following features are preferable:
 - The therapy is of a fixed duration
 - It is standardised by the use of manuals
 - Random assignment is used
 - The patients have a single specifically-selected problem
 - Outcome is measured in symptom counts
- ★ However, given that most therapies do not occur under these conditions in real life, these conditions reduce generalizability; also it is difficult to find an appropriate placebo for psychotherapy.
- ★ The general or average effects of psychotherapy are widely accepted as significant, with variations in outcome more influenced by general severity rather than patient diagnosis, and also by clinician and context-related factors, than by specific treatment 'brands'
- ★ The therapeutic alliance is consistently one of the most important factors in effectiveness of psychotherapy
- ★ The effect of psychotherapy continues beyond the termination of therapy and last longer than non-psychotherapeutic treatments (American Psychological Association 2012, Recognition of Psychotherapy Effectiveness)

Current Evidence-based Recommendations for Psychotherapies

NICE recommendations include:

- ★ **CBT** for people with psychosis/schizophrenia, depression, anxiety disorders, eating disorders, PTSD
- ★ **Interpersonal Therapy (IPT)** for people with depression, eating disorders
- ★ **MBT** for people with personality disorder
- ★ **MBCT** for relapse prevention in depression
- ★ **DBT** for people with personality disorder
- ★ **CAT** for people with depression, anxiety disorders, personality disorders
- ★ **Psychodynamic Psychotherapy** for people with depression, anxiety disorders, PTSD, personality disorder
- ★ **Behavioural psychotherapy** for people with addiction disorders
- ★ **Family/systemic therapy** for eating disorders, psychosis/schizophrenia
- ★ Counselling and supportive psychotherapy: according to patient preference, can be helpful for patients with depression, anxiety disorders, psychosis/schizophrenia, eating disorders, etc., but NOT if another therapy is more strongly indicated

Predictors of response to psychotherapy

The acronym **YAVIS** was coined to indicate that clients improve more in psychotherapy when they are “young, attractive, verbal, intelligent, and successful.” There is no proof that this is true. This idea first appeared in 1964 in the book *Psychotherapy: The Purchase of Friendship* by the US psychologist William Schofield.

To date, the best predictor of outcome in any psychotherapy is the **degree of the therapeutic alliance** (Krupnick et al., 2006). The therapeutic alliance is found to have a significant effect on clinical outcome for *various* psychotherapies and also for active and placebo pharmacotherapy. Ratings of patient contribution to the alliance are significantly related to treatment outcome while the ratings of therapist contribution to the alliance and outcome are not significantly linked. In contrast to the therapeutic alliance, positive perception of therapist throughout treatment (idealization) is not necessary and may even be counterproductive in some cases.

Combining pharmacotherapies and psychotherapies

In a systematic overview of multiple meta-analyses addressing the efficacy of combining psychotherapies and pharmacotherapies, Huhn et al. (2014) compared the effect-sizes of combined therapies against monotherapies of either modality across various psychiatric disorders. While direct head-to-head comparisons were scarce, using the available evidence, the authors concluded that for depression, social phobia, panic disorder and bulimia, the combination therapies produced superior therapeutic effects. The evidence was equivocal for disorders such as schizophrenia and dysthymia where medications were of superior efficacy.

Benefits of combined psycho-pharmacotherapies

- Improved recovery rates
- Faster responses
- Decreased rate of relapse
- Improved long-term social functioning
- Improved medication compliance
- Greater reported satisfaction
- Lower long-term service costs

Challenges in offering combined therapies

- Higher administration costs
- Lack of reliable evidence base
- Practical difficulties in co-administration

The authors also highlighted several difficulties in comparing trials of psychotherapy with pharmacotherapy:

- ★ In general psychotherapy trials are of smaller size; with larger effect sizes when compared to control treatments, especially when the control is a waiting-list option rather than a well-concealed placebo treatment.
- ★ Individual pharmacotherapy trials are more likely to have large sample sizes, blinding, control groups, and intention-to-treat analyses. In contrast, psychotherapy trials have lower dropout rates and have a better quality of follow-up data.
- ★ *Researcher allegiance*: Testing of psychotherapy by its inventors often positively influences the effect size.
- ★ Psychotherapy meta-analyses often miss reporting their authors' conflicts of interest. Interestingly, with psychotherapy studies the conflicts are not restricted to financial benefits while pharmacological trials often received financial sponsorships by the drug industry.

DISCLAIMER: This material is developed from various revision notes assembled while preparing for MRCPsych exams. The content is periodically updated with excerpts from various published sources including peer-reviewed journals, websites, patient information leaflets and books. These sources are cited and acknowledged wherever possible; due to the structure of this material, acknowledgements have not been possible for every passage/fact that is common knowledge in psychiatry. We do not check the accuracy of drug-related information using external sources; no part of these notes should be used as prescribing information

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